



NPHCDA Digital and Innovation Hub

Update call with the DPRS

March 2026

Meeting Agenda

Date: March 26, 2026

Venue: Conference Room, HQ, NPHCDA & Zoom

Duration: 1 Hour (09:00 – 10:00 pm)

Agenda items:



Responsible



Time



1

Opening

Aseeyah

09:00 – 09:05 am

2

Hub Objectives

Aseeyah

09:05 – 09:10 am

3

Progress updates on key deliverables

DIH team

09:10 – 09:30 am

4

Alignment on routine cadence

All

09:30 – 09:40 am

5

AOB/Closing

All

09:40 – 09:00 am

1

Hub objectives

2

Progress updates on key deliverables

1

Hub objectives

2

Progress updates on key deliverables

The PHC Digital and Innovation hub (DIH) coordinates and operationalizes NPHCDA's digitization roadmap to achieve 2 broad objectives

Overall Goal

The goal of this initiative is to **support NPHCDA to deliver actionable data for decision-making and action to increase the effectiveness and efficiency of PHC investments and improve PHC outcomes**

1

Drive programmatic and institutional decision making through data aggregation, advanced analytics, and the deployment of innovative tools

Enhance management, aggregation, and integration of relevant PHC data

Apply advanced analytics and modelling studies to improve PHC programme performance tracking

Support the deployment and scale up of digital solutions for PHC operations and PHC service delivery improvements

2

Establish and institutionalise a centralised hub to facilitate information exchange and establish new data streams through digital innovations

Institutionalise a centralised government-owned coordination mechanism for the implementation of the NPHCDA's data use aspirations and the PHC digitisation road map

Support the development and operationalisation of a digital health tool standardisation and data governance framework that sets minimum requirements and ensures data security

Establish a data repository for warehousing and archiving PHC data for programmatic use and learning

1

Hub objectives

2

Progress updates on key deliverables

The DIH has made progress across key deliverables.../1

Focus		Progress to Date	Details ahead
Digitization	Electronic Community Health Information System (eCHIS)	▪ The deployment of the eCHIS suite is ongoing. The IE module has already been deployed and used for several campaigns, initially across six states and subsequently expanded to fourteen states for the enumeration campaign ▪ Additional modules, including the MNH and Vaccination modules, are scheduled to be deployed in April by Fionet, with five states prioritized for the initial rollout ▪ Fionet has been designated as the primary vendor to deploy the eCHIS suite across 26+1 states, while the remaining ten states will be covered by a local vendor. The Request for Proposals (RFP) for the local vendor has already been issued	
	Electronic Medical Record (EMR)	▪ The EMR use case and workflow is being developed, through desk review of existing frameworks and standards, expert consultation as well as facility visit ▪ Fionet will lead the design and deployment in all BHCPF facilities through a phased rollout before scaling to non-BHCPF facilities ▪ Fionet is also engaging with special duties department to clarify implementation requirements and provide relevant documents to guide the EMR design process	
	Enterprise Resource Planning (ERP)	▪ Delwu conducted an ERP readiness assessment last year and a comprehensive report was shared highlighting the gaps and recommendations for the NPHCDA ▪ KII was also conducted with the across the departments which helped ascertain their needs and offer recommendations to the NPHCDA ▪ The team is seeking to get approval from the ED to initiate the RFP process	

The DIH has made progress across key deliverables.../2

Focus		Progress to Date	Details ahead
Digitization	Data Exchange System	The development of the Data Exchange System to ensure seamless interoperability between the eCHIS and EMR is currently ongoing	
	Analysis and Modelling	- Advanced analytics and modelling studies for PHC to improve priority program performance across e.g BHCPF DFF expansion, Vaccine procurement tracking system have been supported - Periodically produce analytical briefs, reports, and visualizations for technical and executive decision making	
Analytics	Data Management Portal	A governance framework and a data requisition portal has been designed in line with national data protection regulations (NDPR) which has now been hosted on NPHCDA server	
	Dashboard & visualization	Intelligence dashboard to visualize data for CDCU, State report and priority indicators across all programs in real time has been developed	
	Data Warehouse	Mapped key performance indicators to their respective data sources, and currently in progress with developing the centralized data archival system	
	Unified Platform for Data	Plans underway to integrate NPHCDA priority dashboards into one access point (Service delivery, PHC revitalization, and Cash plan dashboards etc)	

Content

Digitization

- 1 Electronic Community Health Information System (eCHIS)
- 2 Electronic Medical Record (EMR)
- 3 Electronic Resource Planning (ERP)

Data and Analytics

- 4 Analytics, Modelling and visualization
- 5 Data Governance Framework and Data Requisition Portal

Content

Digitization

1

Electronic Community Health Information System (eCHIS)

2

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5

Data Governance Framework and Data Requisition Portal

An eCHIS has been prioritized for deployment to address the core needs of the community health workforce program

Decision support nodes

 Priority component

1 eCHMIS Interface - with modules for:

a. Community/Settlement registry module

-  Enumerate families/women/children by name, age, gender and service status
-  Profile communities by terrain, access, geolocation etc.

b. Community service Provision module

-  Log services provided
-  Log referrals
-  Report MPDSR and disease outbreaks (surveillance)
-  Log rumours/infodemics

c. Commodities Management

-  Manage drug and commodities – stock received and dispensed

Make Referrals

Pull Commodities



Clinical decision support

4

PHC Facility EMR Interface (eCHIS tool will also be linked to log-in services provided at health facility level)



Program Management Interface

3

With built in features for routine and campaign decision support for;

i. Microplanning (and macroplanning)

- Target Population in need by service type
- Settlement master lists + coordinates
- Forecast commodity needs to deliver services
- HR requirements (and available)
- Compute financial resources required



ii. Service tracking/monitoring

- Visualize individuals and communities reached
- Service uptake/coverage (?) – individual and aggregates by geography



iii. HW Performance management & payments

- Ascertain/validate visits reported by health worker using geo-tags
- Measure performance of health workers in reaching identified populations
- Digitize and track payments for validated services



2

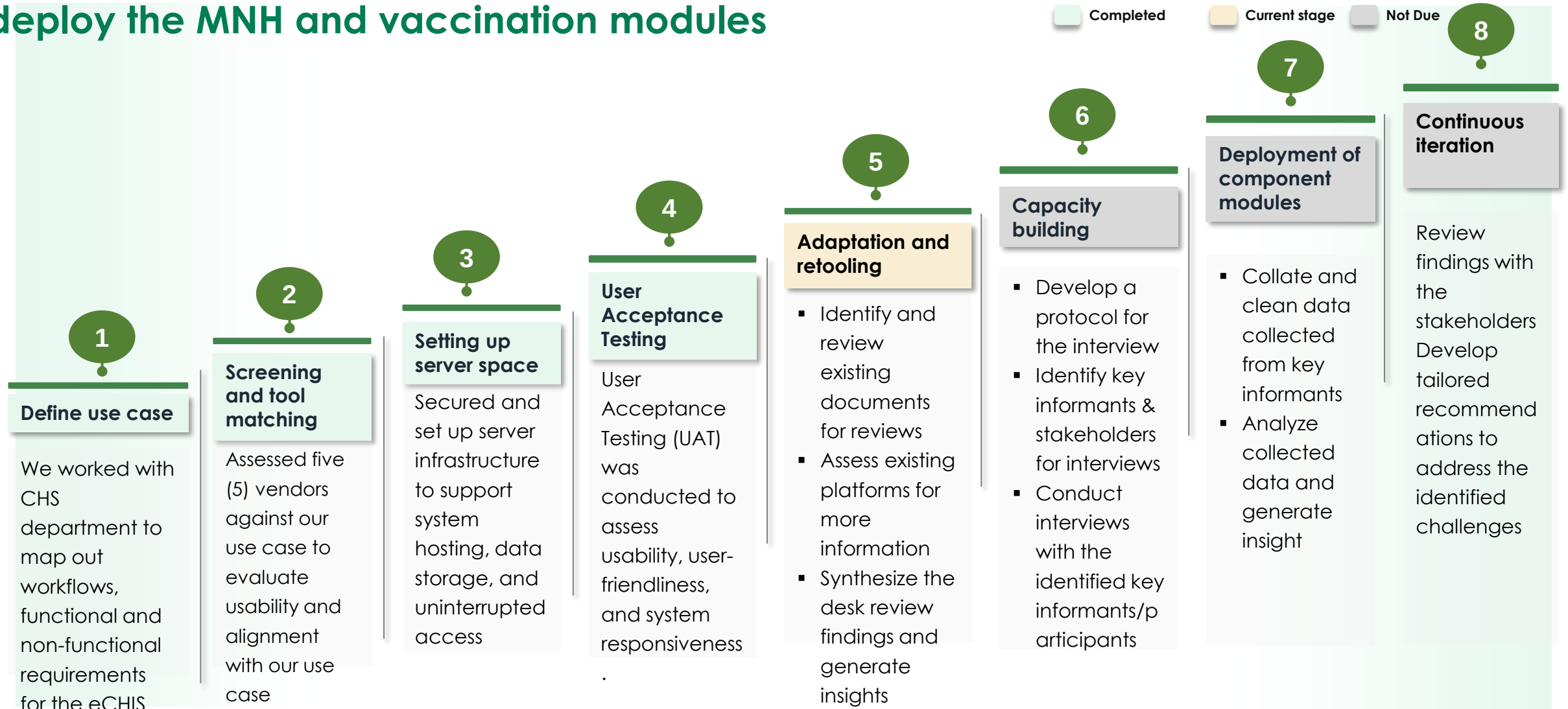
Frontline HRIS

- Enrols HWs by cadre, gender, competency, program and location
- Collects payment details of health workers



Management and planning decision support

IE module has been deployed and tool adaptation is currently ongoing to deploy the MNH and vaccination modules



- So far, the eCHIS has been deployed in 14 states for enumeration and the polio campaign using the Fionet App
- Expansion of the service delivery module to cover all PHC services is ongoing, with deployment tentatively scheduled for April

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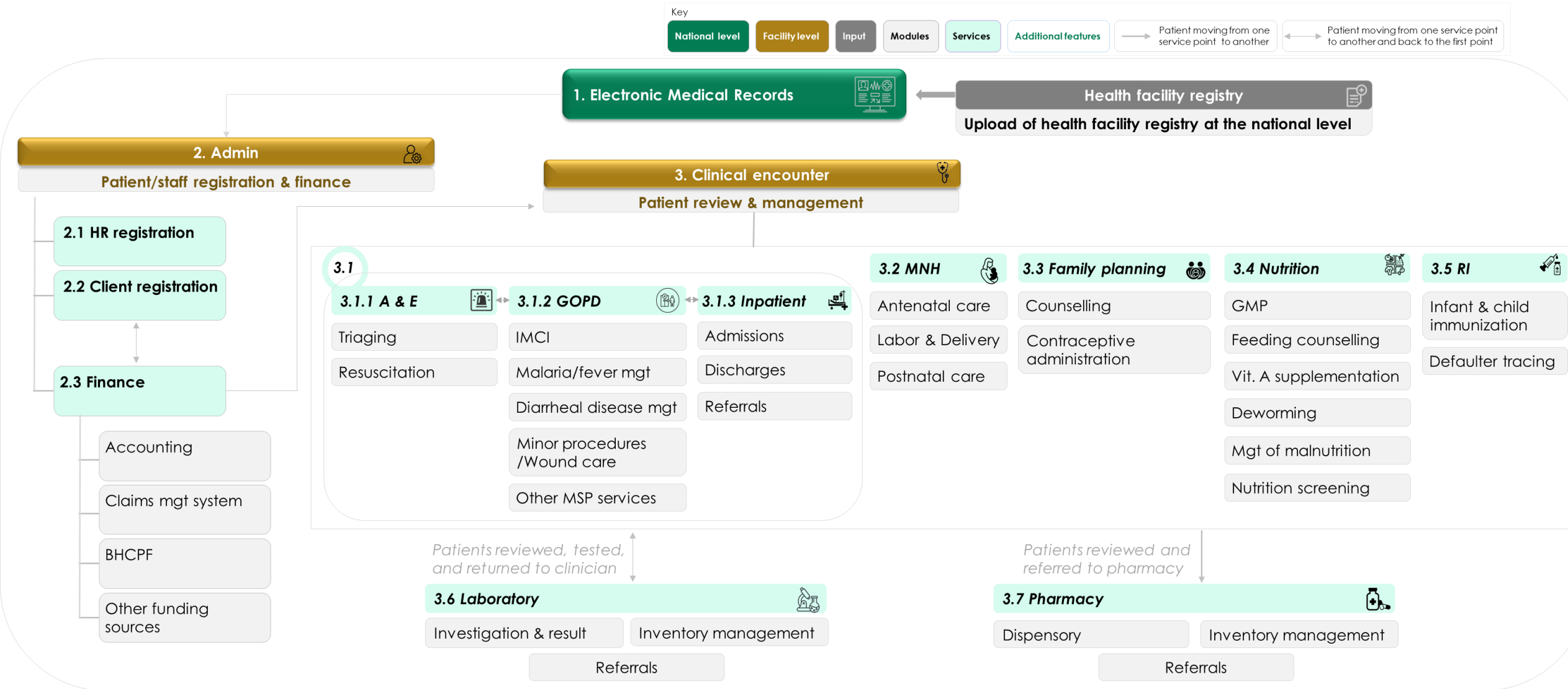
Background

- The National Primary Health Care Agency (NPHCDA) in collaboration with partners has initiated the deployment of the **electronic Community Health Information System (eCHIS)** to strengthen community-level service delivery. However, there remains a critical need for a **complementary clinical documentation system at the primary health facility level**
- The current challenges in **managing individual patient histories, diagnostics, referrals, and treatment plans** are **not fully addressed by community-facing tools**. Therefore, to bridge the gap between community-based data capture and facility-level clinical decision-making, **NPHCDA seeks to design and implement an Electronic Medical Record (EMR) solution tailored for PHC facilities**
- This solution is meant to **align with national digital health priorities** and **integrate seamlessly with eCHIS**, without duplicating its functionality
- This document outlines the methodology, proposed EMR workflow and the core requirements of an EMR

We conducted desk reviews and expert engagements to guide the development of the EMR workflow

Focus	Description	Sources
Desk review 	▪ Conducted desk reviews through observation of EMR demonstration sessions to gain practical insights into expected user interface design and functionality ▪ Reviewed key EMR workflow features to ensure a comprehensive understanding of required system capabilities ▪ Conducted review of the WHO SMART guidelines to ensure alignment ▪ Assessed essential services delivered at the primary health care level to develop an exhaustive list of functional requirements.	▪ EMR vendors websites ▪ Existing videos from previous engagement with EMR vendors ▪ PHC minimum service package ▪ WHO SMART guideline
Expert engagement & validation 	▪ Conducted key informant interviews with; ✓ End users to capture end-to-end workflows and ensure completeness ✓ Facility OICs to validate the workflow and also test practicality at the PHC level ▪ Consulted subject-matter experts to validate the workflows and confirm both operational and technical feasibility at the PHC level	▪ Doctors, pharmacists, and laboratory scientists who currently use or have previously used an EMR ▪ Partners specialized on EMR ▪ BHCPF facilities OICs ▪ Information technology experts

Based on what we found, we developed a proposed Electronic Medical Record Workflow

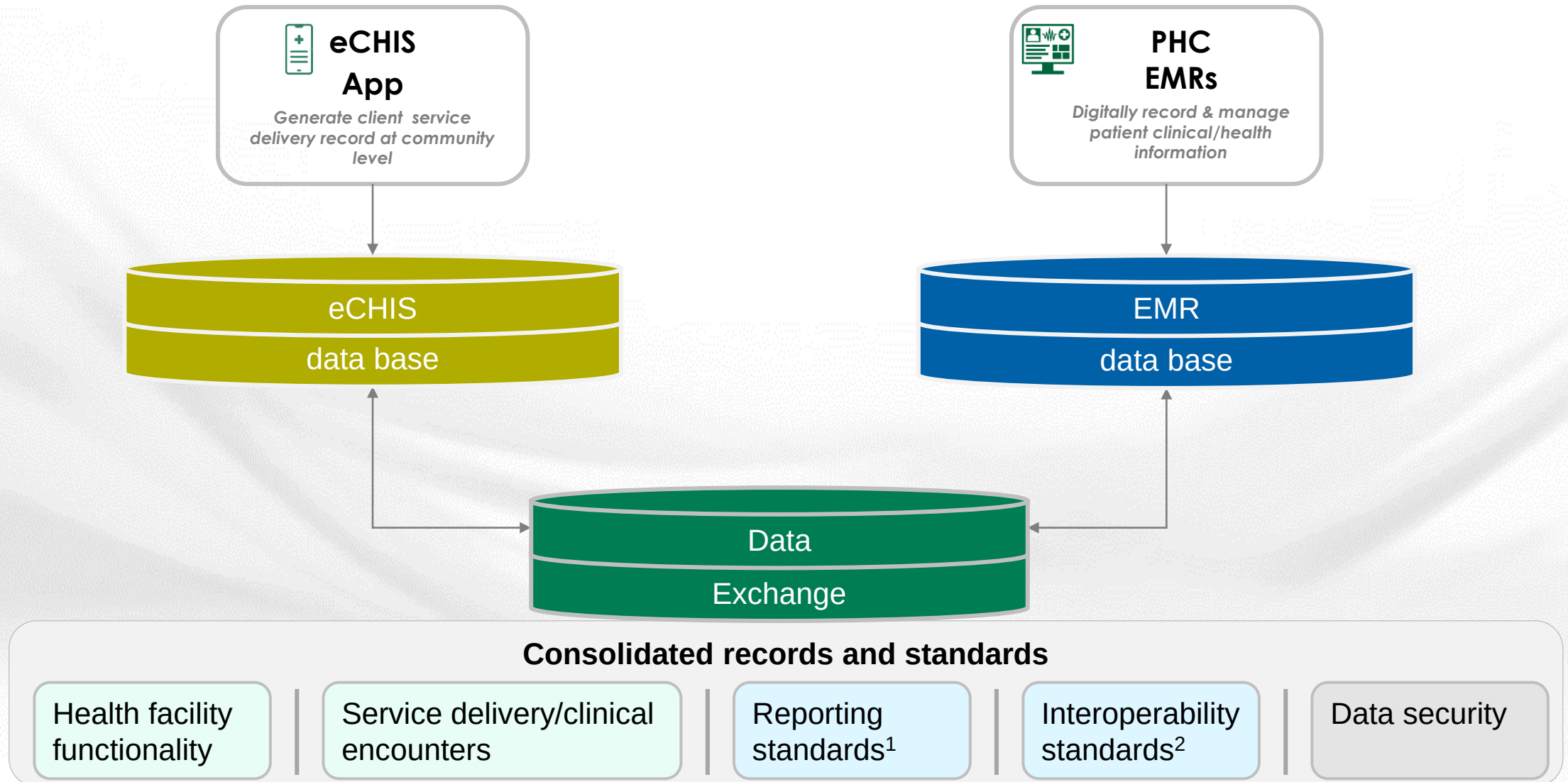


Following deployment, data from PHC EMRs, other interoperable EMRs, and eCHIS will flow into the exchange platform, where it will be systematically mined, refined, and transformed into actionable intelligence by the Data Intelligence Centre

We have identified some core requirements of an EMR System...

Focus		Details
Functional	Admin interface	EMR must capture and manage all patient and staff details which will be captured at the point of registration by admin staff, and also day to day financial management
	Clinical interface	EMR must capture structured clinical documentation (history, diagnosis and documentation)
Non-functional	Adherence to MDS	EMR must capture and manage all data fields listed in the Minimum Data Sets (MDS) for each PHC module
	Interoperability	Must support import/export in open formats (FHIR, CSV, XML)
	Scalability	Modular structure to allow new modules/services to be added as PHC's evolves
	Data Standard	All diagnosis, labs, medications, and immunizations must use standard codes (ICD-10, LOINC, WHO/ATC, CVX, FHIR)
	Security & Privacy	Data encryption, role-based access, and audit trails must be implemented

Following deployment, data from PHC EMRs and eCHIS will flow into the exchange platform



1. Standard data disaggregation (sex, age, location, etc.), Minimum datasets and indicator definitions, District Health Information Software 2 (DHIS2) metadata standards and others 2. Health Level Seven International (HL7), Fast Healthcare Interoperability Resources (FHIR), International Classification of Diseases (ICD-10/11), and others

Source: Team analysis

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Following the directive from the head of civil service, the DIH is working to support the deployment of ERP at NPHCDA

Directive from the Head of the Civil Service of the Federation

“ All MDAs within the Federal Civil Service are to transition to full digital operations and end the use of paper-based processes by December 31st, 2025...(FCSSIP¹, 2021-2025) ”

- In line with this, the Office of the Head of Civil Service of the Federation is accelerating the delivery of **electronic content platforms**² aimed at achieving overall improvement in **information sharing and collaboration through digitization**
- Since the mandate, about 18 federal ministries including the FMoHSW³ have adopted digital platforms such as the **Enterprise Content Management (ECM)** system to automate their workflows

Some digital priorities within the public sector

Human resource

The Office of the Head of Civil Service of the Federation is institutionalizing **Performance Management System** across all federal MDAs

Finance

The Ministry of Finance has operationalized **GIFMIS**⁴ across all government MDFAs for capital budget execution and expenditure management

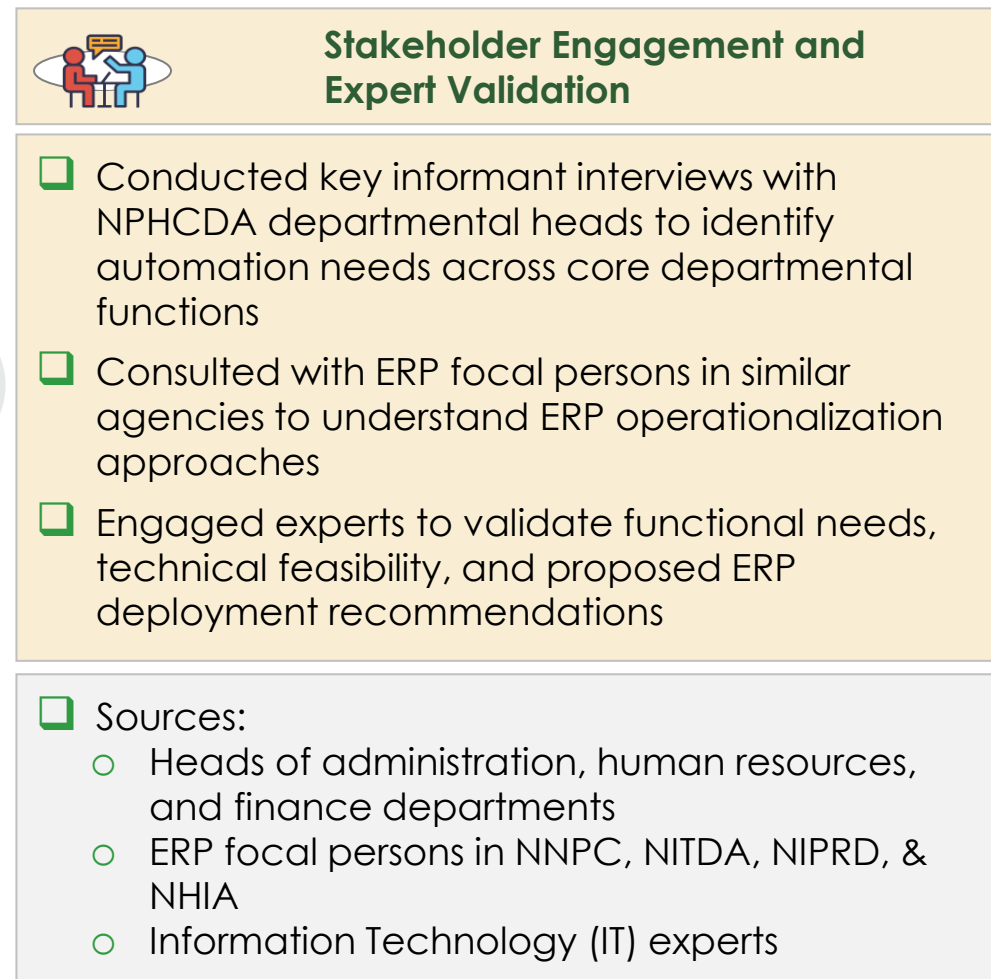
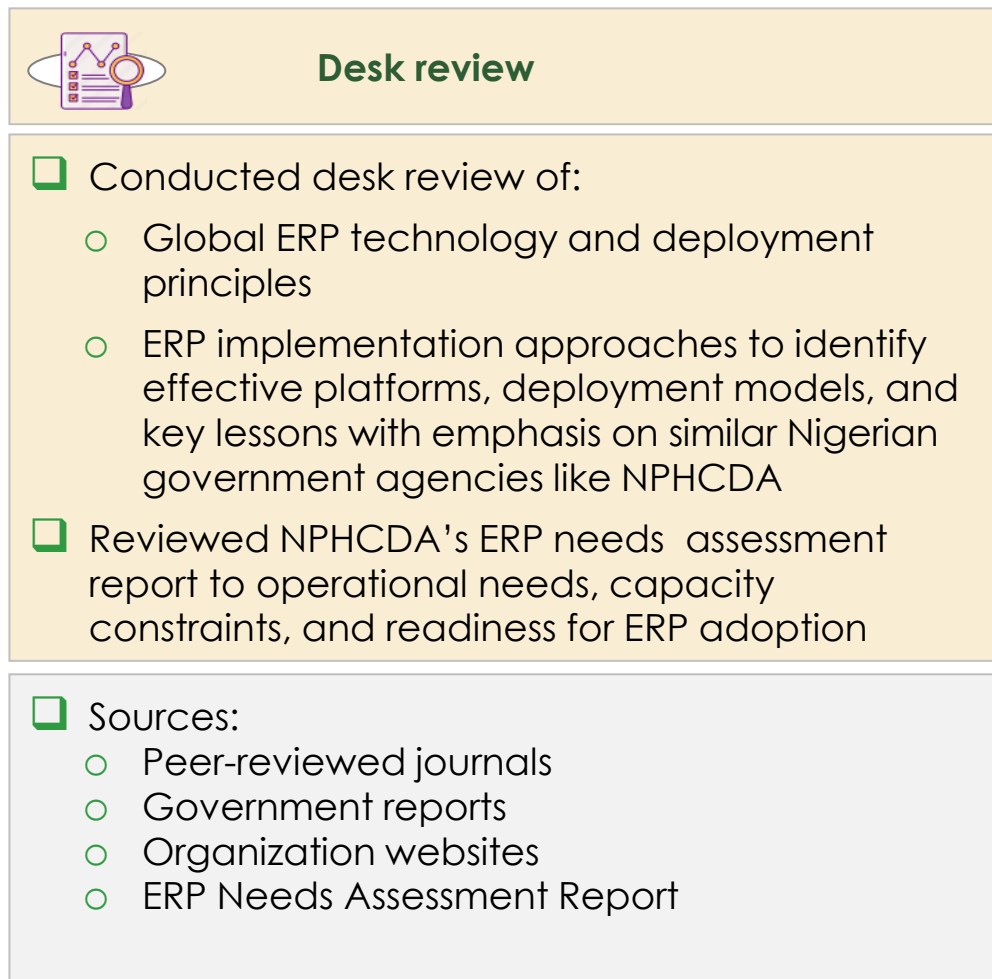
Procurement

The **Bureau of Public Procurement** is developing an ERP platform to digitize and streamline all **phases of public procurement** for the government

1-Federal Civil Service Strategy and Implementation Plan 2-A centralized, enterprise-wide system that manages documents, workflows, and records 3-Federal Ministry of Health and Social Welfare 4-Government Integrated Financial Management Information System

Sources: A-<https://nigeriancivilservicenews.ng/2025/06/12/fg-sets-december-2025-deadline-to-fully-digitise-civil-service-end-paper-usage/> B-<https://ohcsf.gov.ng/2025/09/15/federal-civil-service-commences-strategic-retreat-on-personnel-audit-and-skills-gap-analysis/> C-<https://fmino.gov.ng/bpp-commences-implementation-of-e-government-plan/#:~:text=By,the%20digitalisation%20of%20government%20operations> D-<https://budgetoffice.gov.ng/pdfs/gifmis.pdf> E-<https://gifmis.gov.ng/implementation-update/>

We conducted a structured research and consultation to inform ERP selection and deployment plan for NPHCDA



DATA COLLECTION

The following recommendations were outlined to guide technology and infrastructure considerations for the ERP

Focus	Description	Details ahead
Technology	▪ System Foundation: A modular and scalable software that utilizes open standard-based technologies such as Ex, Java EE / .NET Core ▪ Accessibility: The system must be both web-based and mobile responsive, with offline functionality for certain features where the network is poor ▪ Interoperability: The system should provide support for 3rd party integration with other platforms(e.g., GIFMIS, performance management system) ▪ Licensing: The solution should offer a one-time perpetual licence allowing unlimited users ▪ Hosting: Data ownership with hosting on a hybrid infrastructure (on-prem and cloud server) ▪ Scalability: This should be across the HQ and zonal offices without major technical effort ▪ Maintenance: Vendor-managed maintenance, with progressive transition to NPHCDA for sustainable long-term management ▪ Compatibility: Compatible with both android and IOS devices (phones, tablets and laptops)	
Infrastructure	▪ Hardware: Obsolete laptops should be replaced for core departmental heads and staff with Admin access; (other staff can utilize their phones in the interim). In the long term, laptops should be procured to bridge the identified gaps (15% no computers and 30% obsolete computers) ▪ Power: Back-up power solutions, such as generators and solar systems, should be installed and routinely maintained across both HQ, annex, and zonal offices ▪ Server: Upgrade of existing on-prem server and procurement of cloud storage with capacity to store a large volume of ERP data ▪ Network: Upgrade of the network to 100 Mbps bandwidth at the annex and zonal offices	

Based on the lessons we gathered, we also recommended the following deployment approach for the ERP

Deployment at the HQ Scale up to zones and annexes

Pre-deployment

- Initiate and finalize vendor selection process
- Establish a project management team to oversee the deployment of the ERP system
- Procurement of computer hardware, software, and other relevant infrastructures

Deployment and scale-up approach

Modules	Timelines in Months (M)																	
	M1	M2	M3	M4	M5	M6	M7	M8	M9	M10	M11	M12	M13	M14	M15	M16	M17	M18
Human Resource	APD ² , & Staff welfare			Integration of PMS ¹			Scale up											
Finance & Accounts	Grant Management System & Budget, Expenditure, R &E ass. ³ , &F&F ⁴			Integration of GIFMIS and other payment systems					Scale up									
Procurement	Contract/Grant Mgt, Order Mgt, Logistics & Health Commodities								Scale up									
Administration												Scheduling, Maintenance, Inventory Mgt, Communication						
Internal Audit															Payment & System Audit, Budget Monitoring, Grant Compliance			
Other add-ons/ features: Analytics & archival; Interoperability with other relevant program systems																		

During/Post Deployment

- Institute change management processes to enhance staff adoption
- Provide training and refresher training for newly deployed modules and upgrades of existing modules
- Provide on-site support for immediate troubleshooting and continuous assistance

To ensure the delivery of an ERP solution that aligns with NPHCDA’s objectives, we have outlined the criteria for evaluating and selecting a qualified ERP vendor

Focus	Description
Considerations for Vendor Selection	Prospective vendors are required to: ▪ Be a NITDA-registered indigenous IT service provider with a valid certificate to ensure compliance with regulatory standards ▪ Show proven long-term ERP delivery experience (especially in government or large public institutions) with the capacity to provide a secure and scalable technology ▪ Have professional staff qualifications and competence for the assignment ▪ Have a strong local presence in Nigeria to ensure faster implementation, seamless communication, and a deeper understanding of local concerns ▪ Show clarity and feasibility of the workplan, implementation, roadmaps, and risk management approach ▪ Demonstrate a platform with the following capabilities: ✓ Ease of use, scalability, and adaptability to NPHCDA’s needs ✓ Multi-platform compatibility (mobile and web) ✓ Business process automation and on-demand workflow engine ✓ Integration capabilities to existing platforms, including via API's ▪ Comply with ISO 27001, GDPR, NDPR Act 2023, and NIST Cybersecurity framework ▪ Show strong anonymization, encryption, and access control measures to prevent Personally Identifiable Information (PII) ▪ Demonstrate a clear commitment to a long-term technical support system, updates, troubleshooting, and performance optimization

ISO- International Standard Organization; GDPR- General Data Protection Regulation; NDPR- Nigeria Data Protection Regulation; NIST-National Institute of Standards and Technology; API-Application Programming Interface

Source: Final ERP Needs Assessment Report; ERP Product Requirement Document; lessons learned from literature review; Team Analysis

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The analytics and visualization provide support in tracking various key indicators for different health areas



Health Service Delivery

Basic descriptive analysis and visualization include immunization, malaria, nutrition, antenatal, delivery/postnatal, family planning and facility attendance



Basic Health Provision Fund (BHCPF)

The Team has supported the BHCPF Team with various analytics, including analytics to support facilities added for the expansion



Finance Indicators

The team has also done visualizations for the finance department to enable easy reporting and tracking of financial operations



Operational Indicators

Operational Indicators including human resources for health, revitalization, functionality, media, etc are also included in the visualization and analytics



General Support

The team responds to various data-related needs requested by the agency ensuring a seamless process with data-related asks

The team engaged with key members of the Agency through iterative processes to support all analytics and visualization processes. The analytics have enhanced decision making for various activities within the Agency such as the expansion of BHCPF, etc

The analytics and visualization has followed various steps to arrive at the current output and we still look forward to the implementation of new improvements

Details ahead

Selection of key indicators for display

- Conducted indicator mapping and definition, aligning selected indicators with agency objectives and reporting requirements
- Extracted relevant datasets and established data integration processes to consolidate information from multiple sources
- Validated indicators for display with members of the Agency

Displaying of Visuals on the Dashboard

- The validated indicators were used to create suitable visuals that could support decision making
- The visuals of the indicators were displayed on the dashboard
- The design elements were considered, and the dashboard was deployed with the provision of login details to enable access to the dashboard

Updating and Tracking of Indicators

- The dashboard is updated periodically to ensure up-to-date information is displayed accordingly
- The dashboard is used to track key program indicators at different levels
- Access was also broken down to allow states to visualize and track their indicators and review their performance

The DIH supported the deployment of the NPHCDA service delivery dashboard



NPHCDA
National Primary Health Care Development Agency

[Landing Page](#)

[State Report](#)

CDCU

[State of Health](#)

This dashboard visualizes key performance metrics tracked by the Central Delivery Coordination Unit (CDCU), focusing on the implementation progress of priority government projects and programs across sectors.

States

All

Health Facility Type

BHCPF

[Click to Export](#)

[Back to landing Page](#)

33,531,556

Penta 3 given

7,780

Maternal Deaths

24,260

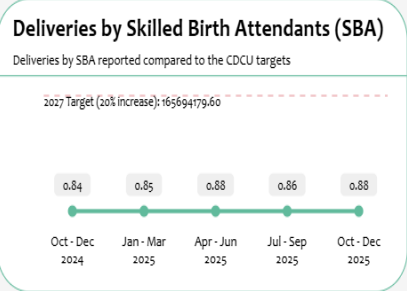
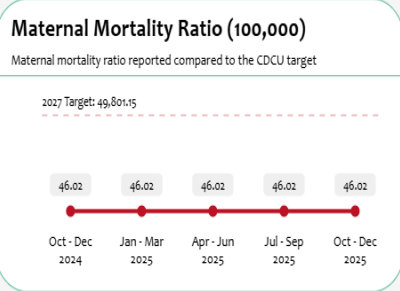
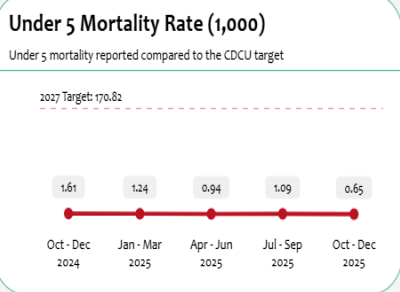
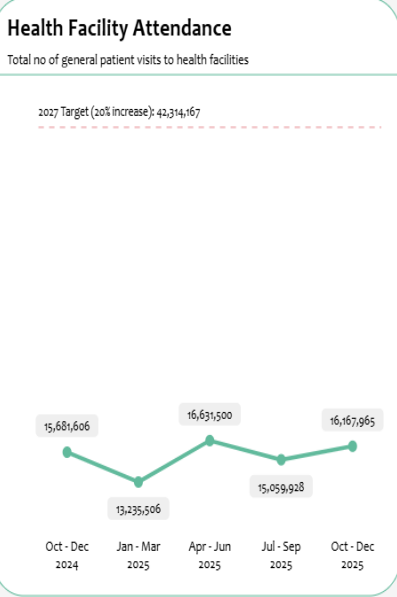
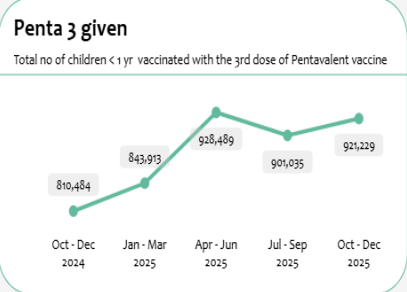
Under 5 Deaths

100%

% BHCPF HF that received their QD

6,916

Frontline Health Workers Trained (Q3 2025)



Data Source: DHIS 2, NDHS 2023



NPHCDA
National Primary Health Care Development Agency

[Landing Page](#)

[State Report](#)

CDCU

[State of Health](#)

This dashboard visualizes key performance metrics on the State of Health, focusing on the implementation progress of priority government projects and programs across sectors.

States

ab Abia State

Zones

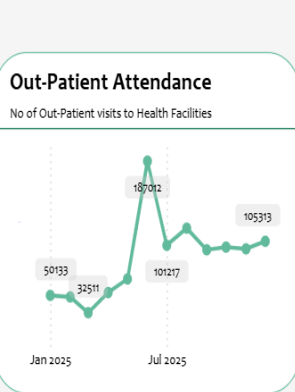
All

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Out-Patient Attendance

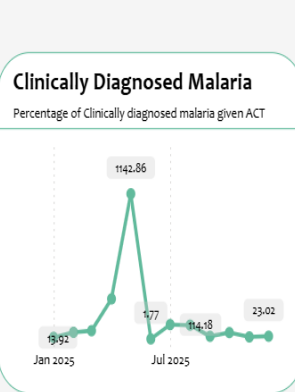
No of Out-Patient visits to Health Facilities



Period	Out-Patient Attendance
Jan 2025	50133
Feb 2025	32511
Mar 2025	187012
Apr 2025	101217
May 2025	105313

Clinically Diagnosed Malaria

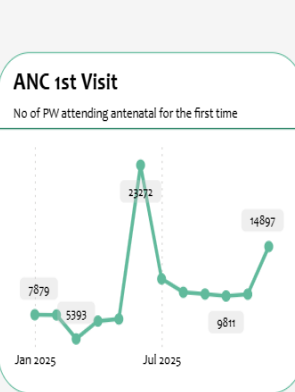
Percentage of Clinically diagnosed malaria given ACT



Period	Clinically Diagnosed Malaria
Jan 2025	19.92
Feb 2025	1142.86
Mar 2025	1.77
Apr 2025	114.18
May 2025	23.02

ANC 1st Visit

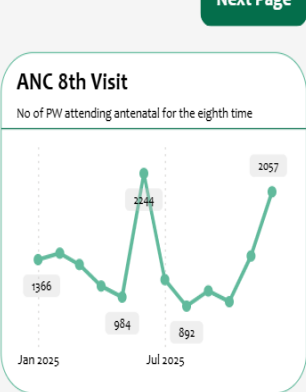
No of PW attending antenatal for the first time



Period	ANC 1st Visit
Jan 2025	7879
Feb 2025	5393
Mar 2025	23272
Apr 2025	9811
May 2025	14897

ANC 8th Visit

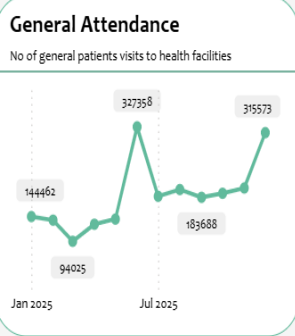
No of PW attending antenatal for the eighth time



Period	ANC 8th Visit
Jan 2025	1366
Feb 2025	984
Mar 2025	2044
Apr 2025	892
May 2025	2057

General Attendance

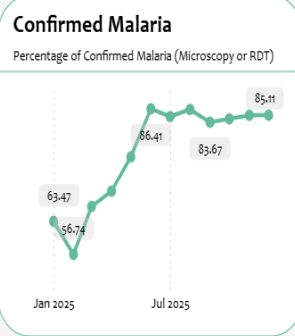
No of general patients visits to health facilities



Period	General Attendance
Jan 2025	144462
Feb 2025	94025
Mar 2025	327358
Apr 2025	183688
May 2025	315573

Confirmed Malaria

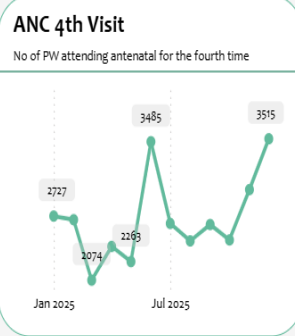
Percentage of Confirmed Malaria (Microscopy or RDT)



Period	Confirmed Malaria
Jan 2025	63.47
Feb 2025	56.74
Mar 2025	86.41
Apr 2025	83.67
May 2025	85.11

ANC 4th Visit

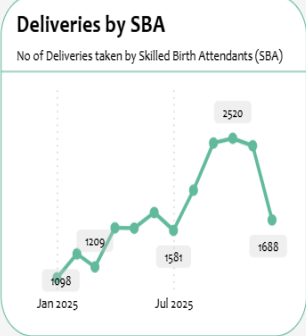
No of PW attending antenatal for the fourth time



Period	ANC 4th Visit
Jan 2025	2727
Feb 2025	2074
Mar 2025	2263
Apr 2025	3485
May 2025	3515

Deliveries by SBA

No of Deliveries taken by Skilled Birth Attendants (SBA)



Period	Deliveries by SBA
Jan 2025	1098
Feb 2025	1209
Mar 2025	1581
Apr 2025	2520
May 2025	1688

Data Source: DHIS 2

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Background

- The NPHCDA has been a key institution in Nigeria's Healthcare Landscape and is tasked with coordinating Primary Healthcare Policies, immunization programs, maternal and child health services, disease surveillance, and health workforce training among other critical functions
- To fulfill these responsibilities effectively, the agency is establishing a digital database of data sets from different initiatives including name-based community registries, service delivery data, health facility assessment information etc
- Therefore, a strong data governance framework is essential for ensuring data security, protecting patient information, and enforcing regulatory compliance for all data collected and used in the process of service delivery and systems optimization
- The DIH aims to ensure proper health digitalization by enhancing data management and interoperability.
- The subsequent slides define the standard data governance framework and methodologies for data requisition, sharing and protection in line with national data protection regulations (NDPR)

For safe data sharing and management, we have defined key data use cases, their underlying rationale, designated requesting party, applicable requirements, and corresponding authorizers

Use Case	Purpose	Requesting party <i>Not-Exhaustive</i>	Authorizers	Applicable req. ¹	Requirements for requesting PII;
Routine Service Delivery	- Enable timely monitoring and reporting of program performance - Support informed decision-making at the field and operational levels	- Federal & state program officers - Implementing Partners	Directors, DPRS, CHS, DCI & PHC-SD	A, B, C, D, E	Requirements for requesting PII; A. Organization Nigeria Data Protection Commission accreditation B. Official letter of request C. Data security and protection plan D. ISO² certification for data systems management E. Signed data sharing agreement F. Ministry/agency project mandate or policy directive G. Ethics committee approval/Ethical clearance from NHREC H. Research protocol summary
Policy Design & Planning	- Use data-driven insights to shape national health strategies - Inform evidence-based budgeting and priority-setting across programs	- Federal & state Ministries of Health - Executives across health agencies - National & State Houses of Assembly Health Committees - Budget & Planning Office - Technical Partners - Professional Bodies & Regulatory Councils - CSOs	- DPRS - ED, NPHCDA	A, B, C, E, F	
National/Global Reporting	- Showcase progress on health indicators to stakeholders - Ensure compliance with national and international obligations (e.g., NDHS, WHO-JRF)	- Development partners e.g WHO, UNICEF, Gavi, etc - National Public Health Institutes (CDC, NCDC) - National statistical offices - National Regulatory Authorities - National Data Repository Regulations	- DPRS - ED, NPHCDA	A, B & E	
Research & Innovation	- Enable evidence-based research, studies, and health publications - Foster data-driven innovations to strengthen PHC service delivery	- Research institutions - National and State Program Officers - Development Partners - Implementing Partners - Academic Researchers - National Public health Agencies	Directors, DPRS, CHS, DCI & PHC-SD	A, C, E G & H	Access Protocols; ➤ Authentication: Multi-factor required for all users ➤ Access Logs: Logged, monitored, reviewed monthly ➤ Data Minimization: Only necessary fields will be exposed per request ➤ Time-Bound Access: Temporary keys revoked after task completion

All data requests will be made using the NPHCDA Data Requisition Portal using **registration and login on data request portal** and no credentials are required for request of non-PII data

We propose a three-phased approach for data requisition and access for the identified stakeholders

Details ahead

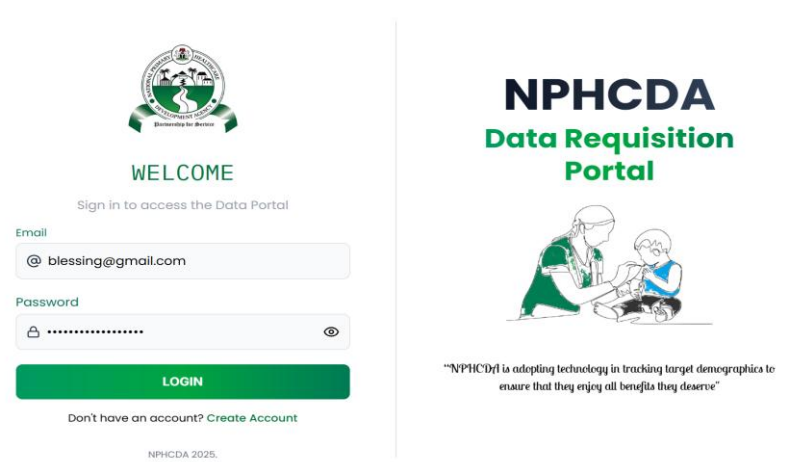
	1 Log a formal Request	2 Screen and approve the request	3 Provide access to data
Description	- Log an official request to NPHCDA's data requisition portal specifying: - Data needs/type - Purpose and scope of data of use - Duration of use - Upload necessary verification documents and credentials (where applicable) - Sign the e-non disclosure agreement (for personal identifiable information data)	- Review submitted credentials for completeness and exhaustiveness - Approve the request (first and second level approval)	Share data to the requesting party in line with data sharing protocol (e.g encrypted/limited access link for PII data)
Duration Responsible	Requesting party (Agency departments, other agencies, state, Partners etc)	- Relevant department heads (Custodians) - Legal/Compliance Team - NPHCDA DIH	NPHCDA DIH
Duration	1 day	1 -2 days	1 day

We recommend the process should not exceed three days to ensure data is used for intended purpose at the right time

A step-by-step process of data requisition, using the NPHCDA data requisition portal (1/2)

Create an account or use already existing credentials to login

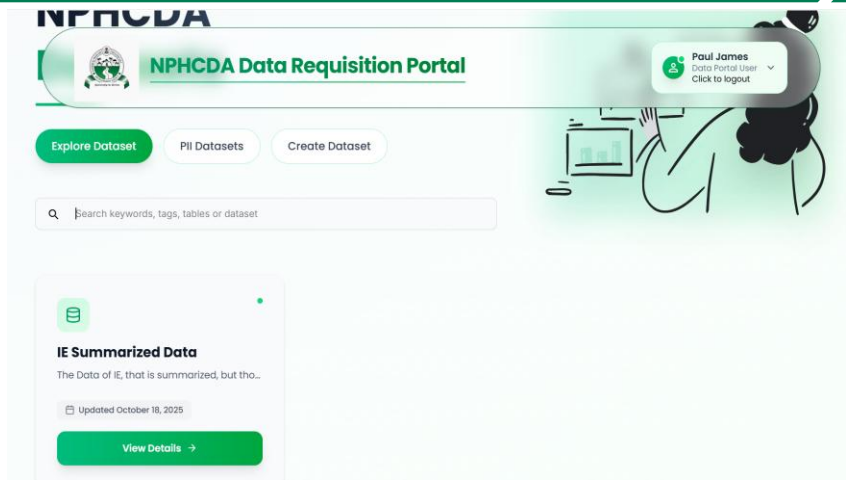
1



The login page features the NPHCDA logo and a 'WELCOME' message. It includes a 'Sign in to access the Data Portal' prompt, an email field with the placeholder 'blessing@gmail.com', a password field, and a 'LOGIN' button. A link for 'Don't have an account? Create Account' is also present.

Click on “view details” to view dataset fields options

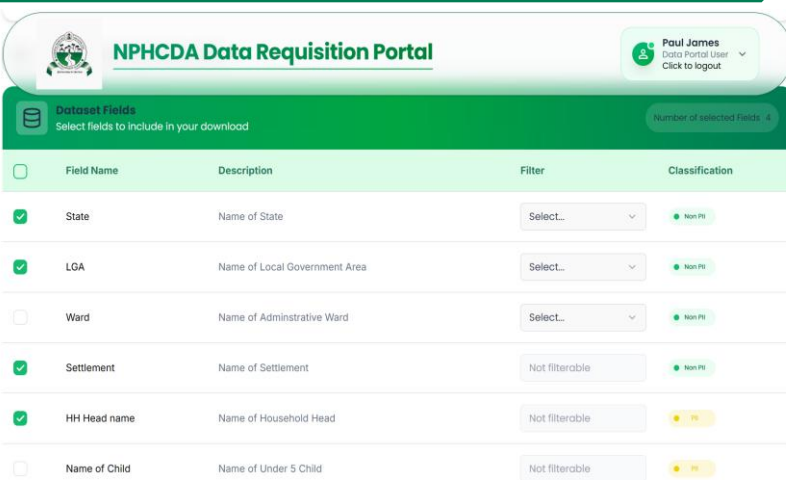
2



This page shows the 'NPHCDA Data Requisition Portal' with a search bar and buttons for 'Explore Dataset', 'PII Datasets', and 'Create Dataset'. A dataset titled 'IE Summarized Data' is highlighted, with a 'View Details' button.

Select required dataset fields

3

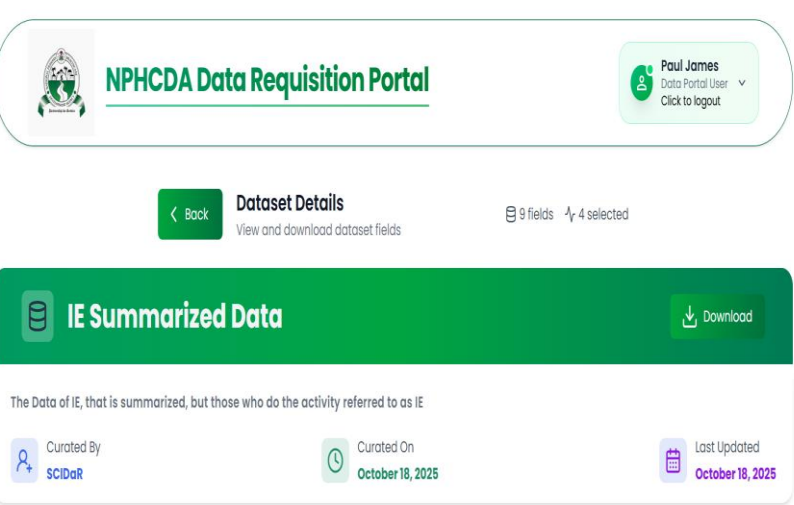


The page displays a table of dataset fields for selection. The table has columns for 'Field Name', 'Description', 'Filter', and 'Classification'. Fields like 'State', 'LGA', 'Settlement', and 'HH Head name' are checked for selection.

Field Name	Description	Filter	Classification
☒ State	Name of State	Select...	Non-PII
☒ LGA	Name of Local Government Area	Select...	Non-PII
☐ Ward	Name of Administrative Ward	Select...	Non-PII
☒ Settlement	Name of Settlement	Not filterable	Non-PII
☒ HH Head name	Name of Household Head	Not filterable	PII
☐ Name of Child	Name of Under 5 Child	Not filterable	PII

Click on download to proceed

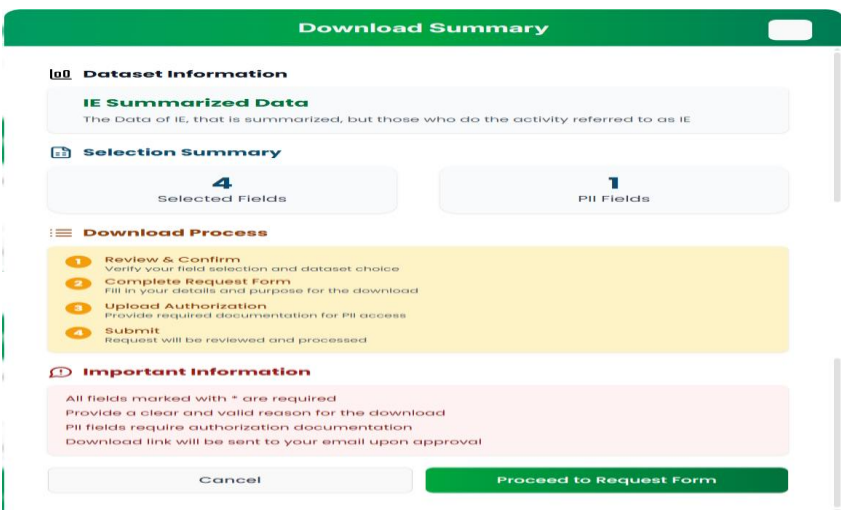
4



The page shows the 'Dataset Details' for 'IE Summarized Data'. It includes a 'Download' button and a 'View and download dataset fields' link.

Click on “proceed to request form” to access the request form

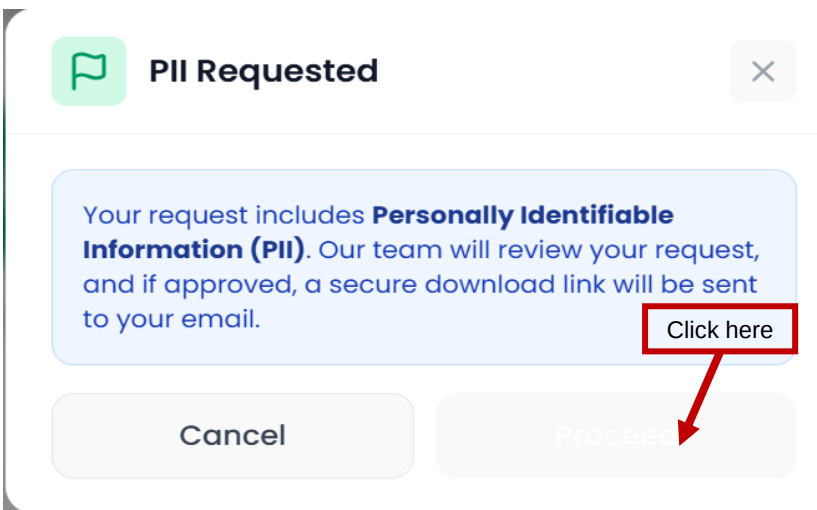
5



The 'Download Summary' page shows the 'Dataset Information' and 'Selection Summary'. It includes a 'Proceed to Request Form' button.

Click here if PII has been requested

6



The modal displays a message: 'Your request includes Personally Identifiable Information (PII). Our team will review your request, and if approved, a secure download link will be sent to your email.' It includes a 'Click here' link and 'Cancel' and 'Proceed' buttons.

A step-by-step process of data requisition, using the NPHCDA data requisition portal (2/2)

Fill in request details

7

Upload required documents

8

Download, sign and upload DSA document

9

Data Request Form

Requester Information

Full Name: Paul James

Organization: SCIDaR

Role: Data Scientist

Request Details

Reason for Data Request: Select a reason...

Description:

Data Request Form

Required Documents

Supporting Documents

Suggested documents for [Implementation Tracking]

- Organization NDPC accreditation
- Official letter of request on organization letterhead
- Government or agency-issued project approval
- Data security and protection plan
- ISO certification for data systems management

You may upload the documents above. DSA is collected in the next section.

Choose document type to upload

Organization NDPC accreditation

Upload file

Data Sharing Agreement (DSA)

Please download the Data Sharing Agreement, have it signed by the head of your organization, then upload the signed copy.

1 Download the Agreement

2 Upload Signed Document

Download DSA Form

Upload Signed DSA Form

File uploaded: DSA.txt

Read terms and conditions and agree to them, then click on "Request Data"

10

Prompt to show request has been sent. An email will be received with the requested data if the request gets approved

11

Terms and Conditions

By proceeding with this request, you agree to:

- Act as the independent data controller and processor
- Protect all data in accordance with data protection regulations
- Use the data only for the stated purpose
- Not share or redistribute the data without proper authorization
- Maintain appropriate security measures for data handling

☒ I have read and agree to the terms and conditions above

Request Data

Request Sent

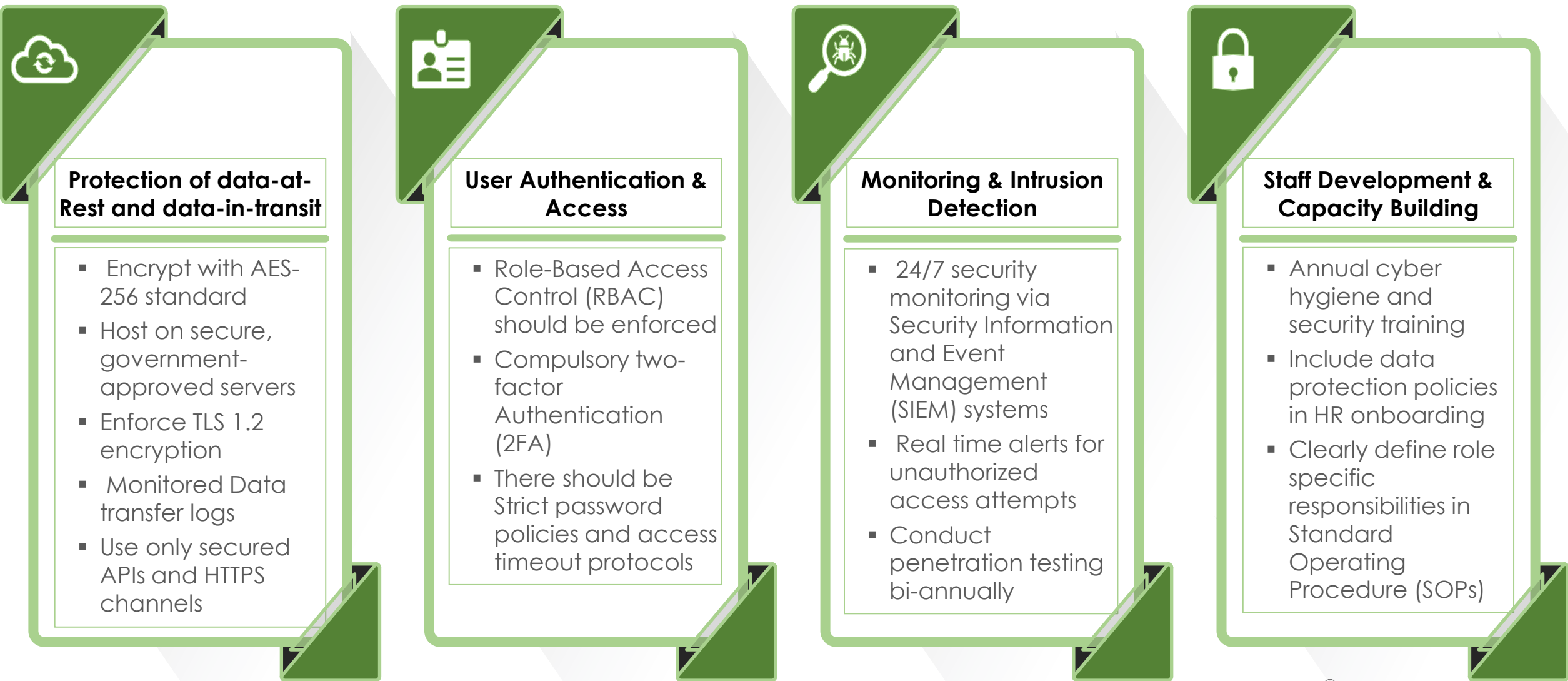
Your request has been received and is pending review because you selected PII data. We will review your documents and send a response to your registered email as soon as we can.

Go to My Request

Established protocols to ensure data confidentiality across all levels will be in accordance with HIPAA regulations

Focus	Description
1 Handling of Personally Identifiable Information (PII)	- Data Sharing Restrictions: PII shall not be disclosed to external entities unless required by national legislation or explicitly approved in writing by an authorized official within NPHCDA - De-identification and Anonymization: Where feasible, data must be de-identified or anonymized before sharing
2 Confidentiality and Data Use Agreements	- All entities (individuals, organizations, or institutions) requesting access to NPHCDA data must execute a confidentiality and data use agreement (<i>see next slide</i>) prior to gaining access. These agreements must: I. Clearly define the approved purpose for which the data will be used (including scope and duration) II. Specify penalties and consequences for misuse or breach of confidentiality
3 Purpose Limitation	- Shared data must only be used for the purposes explicitly stated in the approved request and the corresponding data use agreement - Any secondary use, including the sharing or redistribution of enriched or processed data, is strictly prohibited without prior written consent or a formal amendment to the original agreement
4 Prohibition of Unauthorized Redistribution	- Data recipients are required to implement strict internal safeguards and access controls to ensure that data obtained from NPHCDA is not shared beyond the designated personnel or units specified in the approved request - Internal redistribution of the data whether within departments, teams, or affiliated entities is strictly prohibited without prior written authorization from NPHCDA

Data Protection: The protocols outlined below will safeguard data across the entire health ecosystem



NPHCDA designed a digital roadmap in line with NDHI

